

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 40

Age: 65 years old

Sex: Female

MRN: 000-TEST-40

Admission Date: 06/12/2026

Discharge Date: 06/16/2026

Length of Stay: 4 days

****PRIMARY DIAGNOSIS****

Total hip arthroplasty (THA), left hip — elective procedure for end-stage osteoarthritis with cemented prosthesis placement.

****SECONDARY DIAGNOSES****

- Rheumatoid arthritis (moderate, on DMARDs)
- Gastroesophageal reflux disease (GERD)
- Hypertension
- Type 2 diabetes mellitus

****HOSPITAL COURSE****

Patient 40 presented for elective left THA on 06/12/2026. Pre-operative labs and imaging were unremarkable. Procedure performed without complications. Cemented prosthesis (acetabular and femoral components) placed with standard anterolateral approach. EBL

approximately 300 mL. Patient extubated in PACU. Post-operative course uncomplicated. Pain well-controlled with multimodal analgesia. Ambulated with walker on POD #1; transferring independently with contact guard assist by POD #2. Wound healing well, no signs of infection or hematoma formation. DVT prophylaxis initiated per protocol. PT/OT cleared for discharge with outpatient follow-up.

Key events:

- POD #0–1: Incentive spirometry, ankle pumps, sequential compression device
- POD #2: Advanced to ambulation with assistive device; bed mobility independent
- POD #3: Stairs negotiated with PT supervision; hip precautions reinforced
- Drain removed POD #2; dressing changed to Band-Aid

****MEDICATIONS AT DISCHARGE****

| Medication | Dose | Frequency | Duration |

|---|---|---|---|

| Enoxaparin (Lovenox) | 40 mg | SC daily | 14 days |

| Acetaminophen | 650 mg | PO q.6h. PRN pain | 2 weeks |

| Oxycodone IR | 5 mg | PO q.4h. PRN moderate-severe pain | 10 days; f/u adjust |

| Pantoprazole | 40 mg | PO daily (morning) | Continue indefinitely |

| Methotrexate | 15 mg | SC weekly (Tuesday) | Continue; do NOT d/c |

| Lisinopril | 10 mg | PO daily | Continue |

****DISCHARGE CONDITION****

Stable. Vital signs within normal limits. Pain controlled at 3/10 at rest.

****ACTIVITY & WEIGHT-BEARING STATUS****

****Touchdown Weight-Bearing to Partial Weight-Bearing (TTWB→PWB)**** on left lower extremity for 2 weeks, then advance to Weight-Bearing As Tolerated (WBAT) as tolerated per PT.

Continue walker or crutches (no single-point cane) until cleared by PT (anticipated 4–6 weeks post-op).

****HIP PRECAUTIONS — NO EXCEPTIONS:****

- No hip flexion >90 degrees (no low chairs, toilets, car seats without cushions)
- NO internal rotation of hip
- NO hip adduction past midline
- Keep pillow between legs when lying down
- Modified shoes, reacher for floor objects
- Follow elevated toilet seat & firm chair protocol at home

****WOUND CARE****

Incision clean, dry, intact. Sutures remain in place; removal scheduled at first follow-up visit. Patient to keep dressing clean and dry. Showering permitted with waterproof covering over inc